

Suicide Attempts and Associated Factors in Newfoundland and Labrador, 1998–2000

Reza Alaghebandan, MD¹, Kayla D Gates, MSc, PhD Candidate²,
Don MacDonald, MSc, PhD Candidate³

Objective: This study examined the epidemiology and associated factors for suicide attempts requiring hospitalization in the province of Newfoundland and Labrador.

Method: We extracted data from the provincial hospital separation database. Outcome measures included incidence rates (IRs) of suicide attempts by age, sex, and geographical region of residence. We also analyzed sociodemographic data to determine associated factors.

Results: A total of 978 patients who were hospitalized owing to suicide attempts were identified for 1998–2000, giving an overall IR of 68.7 per 100 000 person-years (P-Y). The age-specific rate for people aged 15 to 19 years was much greater, at 143.0 per 100 000 P-Y. The overall female-to-male ratio was 1.3, with an attempted suicide rate of 76.1 per 100 000 P-Y for female patients and 60.3 per 100 000 P-Y for male patients ($P = 0.001$). Labrador (210.2 per 100 000 P-Y), a region with a high Aboriginal population, had a higher rate of suicide attempts, compared with the island portion of the province (59.0 per 100 000 P-Y) ($P < 0.001$). More than 70% of hospitalizations were associated with psychiatric diagnosis. Poisoning was the most frequent method of attempting suicide. Higher IRs of suicide attempts were found among people who were divorced or separated and among those who were less educated ($P < 0.001$).

Conclusions: Suicide attempt represents a significant public health concern in the province, particularly in Labrador. An increased risk of suicide attempts was associated with single status, female sex, younger age (teen or young adult), and low educational level during the index attempt. Further research is needed to explicate these findings and increase our understanding of attempted suicide.

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Clinical Implications

- This is the first comprehensive population-based study of the epidemiology of suicide attempts requiring hospitalization in Newfoundland and Labrador that reports rates, trends, and associated factors.
- Young age, female sex, single or divorced marital status, Aboriginal background, and low level of education were associated with high rates of suicide attempts.
- Our study provides the opportunity for researchers to expand on its findings.

Limitations

- The provincial hospital separation database does not include cases of attempted suicide that resulted in outpatient treatment in hospital emergency rooms or other medical facilities. Thus patients hospitalized owing to suicide attempt cannot be considered representative of all persons who deliberately harm themselves in the overall community.
- Attempted suicide that does not result in serious injury is usually treated in the community. Many individuals do not see health professionals but are helped by family or friends or perhaps by no one at all. Therefore, such events could not be counted in this study.
- We used ICD-9 codes to determine the definitions used for suicide attempts resulting in hospitalization. This may overestimate the number of hospital discharges for suicide attempts, because self-inflicted injuries specified as intentional, but without a suicidal intent, are included.

Key Words: *suicide attempts, epidemiology, hospitalization, Newfoundland and Labrador, Canada*

Suicide is an action, not an illness. It is an important and preventable public health problem and a considerable drain on resources in both primary and secondary health care settings. In 1999, approximately 4074 Canadians took their own lives, an average of about 11 suicides daily (1). Canadians are 7 times more likely to die from suicide than to be victims of homicide (2,3). Between 1993 and 1998, suicide claimed considerably more lives than did motor vehicle accidents (3,4). Suicide has, in fact, remained a “hidden epidemic.”

The province of Newfoundland and Labrador has consistently been found to have one of the lowest official rates of suicide in Canada, which may be the result of underreporting (5–7). The history of low rates of suicide in Newfoundland and Labrador should not be considered accurate unless adequate information through further research can be obtained. The available data on suicide and suicide attempts in Newfoundland and Labrador are limited, and only a few studies have been conducted in the province on suicide during the last decade. This study was carried out to investigate the epidemiology of attempted suicide resulting in hospitalization and associated factors in Newfoundland and Labrador. The objective of this study was to examine patterns and associated factors for suicide attempts requiring hospitalization in the province. We hope that this study will contribute to the development and targeting of suicide prevention efforts in Newfoundland and Labrador to protect the population at risk.

Methods

We drew data on hospitalizations related to suicide attempts from January 1998 to December 2000 from the CDMS (the provincial hospital separation database) maintained by the NLCHI. The information in the CDMS comes from the discharge summary completed by physicians at all facilities at the end of each uninterrupted patient stay. The file contains data on all acute inpatient cases that were separated from provincial health care facilities.

Abbreviations used in this article

ANOVA	analysis of variance
CDMS	Clinical Database Management System
CI	confidence interval
IR	incident rate
NA	not available
NLCHI	Newfoundland and Labrador Centre for Health Information
P-Y	person-years

Hospitalizations related to suicide attempts were defined as the presence of the ICD-9 (8) codes E950 to E959 for a patient discharged alive (all deaths attributed to completed suicide were excluded). This ICD-9 category includes injuries resulting from deliberate self-harm, acts of intentional self-poisoning, or injuries irrespective of the apparent purpose of the act (9). This may overestimate the number of hospital discharges for suicide attempts, since self-inflicted injuries specified as intentional, but without a suicidal intent, are included. Because some patients do not have serious intent to kill themselves, the term “attempted suicide” may not apply to all deliberate self-harm cases; however, we used the term for the purpose of this study.

Since hospitalizations for suicide attempts are rare among young children, the analysis pertains only to patients aged 10 years or over. Hospitalization rates for attempted suicides were calculated as the number of individuals, aged 10 years or over, who attempted suicide one or more times in a calendar year, divided by the corresponding population estimates, and multiplied by 100 000. We calculated age-specific rates as the number of individuals who attempted suicide in each age group, divided by the corresponding estimated population, multiplied by 100 000. IRs are presented per 100 000 P-Y, which is defined as an estimate of the time-at-risk in years of all people in this study.

We used SPSS (10) and EpiInfo (11) to conduct the analyses. We used the chi-square test to compare categorical data and ANOVA to compare means. The level of significance was set at 0.05. Because comparisons between IRs may reflect random variation rather than real differences, we calculated 95% CIs to assess the variation of incidence and hospitalization rates.

Results

There were 1174 attempted suicide-related hospitalizations (978 unique individuals) in Newfoundland and Labrador during 1998 to 2000. Table 1 shows hospitalization rates and IRs for attempted suicides for each year. Both hospitalization rates and IRs of attempted suicide significantly increased during the study period ($P < 0.001$).

Table 2 presents the distribution of patients according to age group and sex. The mean and median ages were 33.2 and 33.0 years, respectively, with an SD of 14.5 years. The mean age of male subjects (35.1 years) was greater than that of female subjects (31.8 years) ($P < 0.01$). The overall female-to-male ratio for attempted suicide was 1.3, with a ratio of 0.84 among the oldest (aged 50 years and over). IRs of attempted suicide among both male and female subjects significantly increased during the study period ($P = 0.007$ for male subjects and $P < 0.001$ for female subjects). The age-specific incidence rate curve of attempted suicide in Newfoundland and

Table 1 Hospitalization and Incidence rates of suicide attempts, Newfoundland and Labrador

Year	Number of hospital separations	Hospitalization rate* (95%CI)	Number of unique patients	IR** (95%CI)
1998	354	74.0 (66.6–82.2)	283	59.2 (52.6–66.6)
1999	369	77.8 (70.2–86.3)	300	63.3 (56.4–70.9)
2000	451	95.7 (87.2–105.1)	395	83.8 (75.9–92.6)
Total	1174	82.4 (74.5–91.1)	978	68.7 (61.5–76.7)
* Rate per 100 000 population				
** IR per 100 000 P-Y				

Labrador is “bimodal,” with one peak among patients aged 15 to 19 years and a second peak at age 35 to 39 years.

Rates of attempted suicide for separated or divorced (114.0 per 100 000 P-Y) and single (109.0 per 100 000 P-Y) people were significantly higher than for married or common law people (27.2 per 100 000 P-Y) ($P < 0.001$). The incidence rate of suicide attempts among those aged 15 years or over who had fewer than 12 years of education (37.2 per 100 000 P-Y) was significantly higher than for those with 12 or more years of education (15.0 per 100 000 P-Y) ($P < 0.001$). The rate of suicide attempts among single persons in Labrador (290.9 per 100 000 P-Y) was higher than the rate for island portion of the province (98.7 per 100 000 P-Y) ($P < 0.001$).

In this study, poisoning by solid or liquid substances was the most common method of suicide attempt (87.0%), followed by cutting (7.1%) and hanging (2.0%). Violent methods such as cutting and firearms use were more common among male than among female subjects (11.4% vs 5.6%) ($P < 0.001$). Rates of hanging and firearms use were significantly higher in Labrador (13.2%) than in the island portion of the province (1.7%) ($P = 0.001$).

Of the 1174 hospitalizations, 831 (70.8%) were associated with a mental disorder diagnosis requiring hospitalization. The most common were psychotic-related disorders (for example, manic-depressive psychosis and schizophrenia) (27.7%) and adjustment reaction (27.7%). In this study, adjustment reaction (151 of 482, 31.3%) was the most common type of associated mental disorder among female subjects, while the most frequent among male subjects was psychotic-related disorders (101 of 349, 29.0%). The rates of associated mental disorders for Labrador and the island portion of the province were 29.5% (38 of 129) and 75.8% (639/843), respectively ($P < 0.01$).

The rate of suicide attempts was significantly higher in Labrador than the island portion of the province (210.2 vs 59.0 per 100 000 P-Y) ($P < 0.001$). Rates for male and female subjects in Labrador were also significantly higher than those for the

island portion of the province (187.3 vs 52.5 per 100 000 P-Y for male subjects and 242.2 vs 65.2 per 100 000 P-Y for female subjects) ($P < 0.001$). Rates of attempted suicide also increased significantly during the study period for both geographic areas ($P < 0.01$ for the island portion of the province and $P = 0.009$ for Labrador).

The mean age of those who attempted suicide in Labrador (mean 26.6, SD 11.2 years) was significantly lower than for the island portion of the province (mean 34.3, SD 14.7 years) ($P < 0.01$). The mean age of female subjects was significantly lower than that of male subjects for the island portion of the province (34.3 vs 36.4 years) ($P < 0.01$). Both male and female patients were significantly younger in Labrador than in the island portion of the province ($P < 0.001$ for male patients) ($P < 0.001$ for female patients). Age-specific IRs of suicide attempts among the population aged 15 to 24 years on the island portion of the province and Labrador were 59.0 and 557.0 per 100 000 P-Y, respectively ($P < 0.001$). No suicide attempts were found for the elderly (that is, those aged over 65 years) population in Labrador.

Discussion

Most epidemiologic research has addressed completed suicide, since data on deaths resulting from suicides are systematically gathered. Attempted suicide is more difficult to study, because there are generally no accepted reporting procedures; provinces and territories do not gather systematic data on suicide attempts, and many nonlethal attempts are not identified as such. To our knowledge, this is the first report of attempted suicide in Newfoundland and Labrador and one of the most comprehensive studies of suicide attempts in Canada.

Rates

The hospitalization rate for suicide attempts in Newfoundland and Labrador was 82.4 per 100 000 P-Y, which is lower than the national rate of suicide attempts ($P = 0.046$) (3). However, the overall hospitalization rates for attempted suicide are higher for Newfoundland and Labrador than all the rates for New Brunswick and Quebec (3).

Table 2 Age-specific IR of suicide attempts by sex, Newfoundland and Labrador

Age group (years)	Total	IR ^a (95%CI)	Male		Female		<i>P</i> ^b
			Number	IR ^a	Number	IR ^a	
10 to 14	55	49.2 (37.4–64.6)	14	24.5	41	75.0	< 0.001
15 to 19	179	143.0 (123.1–165.9)	63	98.6	116	189.1	< 0.001
20 to 24	101	89.0 (72.9–108.6)	47	81.6	54	96.7	0.39
25 to 29	80	74.0 (59.0–92.6)	39	72.9	41	75.0	0.9
30 to 34	100	82.2 (67.2–100.4)	50	84.1	50	80.3	0.8
35 to 39	130	97.5 (81.7–116.1)	51	77.8	79	116.4	0.02
40 to 44	110	81.8 (67.5–98.9)	57	85.9	53	77.7	0.6
45 to 49	89	68.5 (55.3–84.7)	32	49.5	57	87.2	0.009
50 to 59	88	45.3 (36.5–56.1)	46	47.1	42	43.4	0.7
60 to 74	33	19.3 (13.5–27.5)	19	22.7	14	16.0	0.3
≥ 75	8	9.9 (NA ^b)	5	15.6	3	6.2	0.3
Total	973 ^c	—	423	60.3	550	76.1	< 0.001

^aIR per 100 000 P-Y^bcomparing rate of suicide attempts between male and female subjects in each age group^cAge was unknown for 5 patients

The IR of attempted suicide was 68.7 per 100 000 P-Y in Newfoundland and Labrador, which is lower than the national rate. According to Langlois and Morrison (3), approximately 20 000 individuals were hospitalized in Canada for attempted suicide and discharged alive in 1998–1999, giving an IR of 76.0 per 100 000 P-Y ($P = 0.045$ for comparison between provincial and national rates).

Our results show that the rate of suicide attempts in Labrador, a region with a high Aboriginal population, was significantly higher than the rate in the island portion of the province and for Canada as a whole. Thirty-four percent of the population in Labrador is Aboriginal. Although the CDMS, like most administrative databases, does not capture ethnic background, we believe that the high rate of suicide attempts in Labrador can be attributed, at least partly, to the Aboriginal population. Further studies are needed to assess suicide attempts and associated factors among the Aboriginal population in Labrador. Aboriginal people in both Canada and the US appear to be particularly vulnerable to suicide, with rates exceeding those in the general population (12–14). Suicide rates among the Aboriginal population are 3 to 6 times the rate of the national average, depending on the community under study (15). Many reasons have been proposed for this, including socioeconomic disadvantage, geographic isolation, rapid culture change with attendant acculturation stress, poverty, alcohol abuse, family violence, access to firearms, and oppressive effects of a long history of internal colonialism (14,16–21).

Attempted suicide rates significantly increased from 59.2 to 83.8 per 100 000 P-Y ($P < 0.001$) in Newfoundland and Labrador during the study period. These findings call on

society, health professionals, and policy-makers to recognize suicide as an increasing and persisting challenge. Similarly, the IR of suicide attempts in Labrador nearly doubled from 1998 to 2000, which is an alarming sign for the Aboriginal population, as well as for authorities at the regional, provincial, and national level.

Age

In this study, the rate of suicide attempts was found to be highest among those aged 15 to 19 years. Earlier studies have suggested that preadolescents and adolescents are at higher risk of suicide in Newfoundland and Labrador (5). Reports from Health Canada and Statistics Canada show a similar pattern of suicide attempts with respect to age distribution (3,15). Among the population aged 15 to 24 years, the attempted suicide rate for Labrador (557.0 per 100 000 P-Y) was nearly 6 times that for the island portion of the province (95.0 per 100 000 P-Y). Malchy and others (12), in a study of suicide among Manitoba's Aboriginal people, showed that the rate of suicide among Aboriginal adolescents was nearly 7 times that of non-Aboriginal adolescents. These findings suggest that Aboriginal adolescents and youth are at greater risk than are the non-Aboriginal population. In general, youth suicide is a tragic event that relates partly to events associated with this life stage. Resolving the challenges that are part of youth development, such as identity formation and gaining acceptance from families, can be stressful for young people. For example, loss of a valued relationship, interpersonal conflict with family and friends, and the perceived pressure for high scholastic achievement can be overwhelming. For those who are vulnerable to suicide, these developmental stressors can

create a serious crisis for which suicide may seem to be the only solution. The impulsiveness of youth and their lack of experience in dealing with stressful issues also contribute to the higher risk of suicide (15).

An interesting finding in this study was that no suicide attempts were recorded for the elderly (that is, those aged over 65 years) in Labrador. This may be due to the fact that the elderly attempt suicide less often than do younger people but are successful more often. Malchy and colleagues reported similar results among Manitoba's Aboriginal people. This lack of recorded suicide attempts (12) may also reflect cultural factors or some other protective effect conferred by aging that is unique to this population, which deserves further investigation.

In this study, the rate of suicide attempts among female subjects aged 10 to 19 years was significantly higher than among male subjects ($P < 0.001$). These figures parallel results from the 2002 Canadian Community Health Survey, which found that young adolescent girls are more likely than boys to have suicidal thoughts (22). Health Canada reported that, in the 2 youngest age groups of female subjects (that is, those aged under 15 years and between 15 and 24 years), hospitalization rates for attempted suicide decreased from 1995 to 1999 (15). Conversely, our results show that hospitalization rates for suicide attempts among female subjects in Newfoundland and Labrador increased from 1998 to 2000. Pinhas and others showed that gender-role conflict is an important factor associated with significant distress in young girls' lives. Adolescence is a time when girls take on new gender-specific roles. Developmentally, adolescence is a time for exploration and individuation (23), and yet data suggest that, for some girls, it is a time when options constrict. They may be forced to choose between parents' traditional expectations and their own more contemporary views. Young women exposed during adolescence to contradictory societal and familial role expectations may find themselves with conflicting desires (23).

Sex

In Newfoundland and Labrador, female subjects had a higher rate of attempted suicide than did male subjects ($P = 0.0003$), which is also the case in Canada, the US, and Europe (3,15,24,25). In a similar study in 1999–2000, female subjects were 1.5 times more likely than were male subjects to be hospitalized owing to attempted suicide in Canada (15). Some studies have suggested that women are more likely than men to make suicide attempts that are actually intended to be nonfatal, but this view remains controversial (3,26,27). An increasing trend in suicide attempt rates was seen among both sexes during the study period and was more significant among female subjects than among male subjects. Moreover, female

subjects tend to be at greater risk for attempting suicide than male subjects, and this risk increased over the study period.

Marital Status

When we consider marital status, divorced or separated persons were found to have the highest rate of suicide attempts. The suicide attempt rates for single, divorced, or separated people were 4 times higher than rates for the married or common-law population. The high rate of suicide attempts among single people may be due to the fact that the peak age for suicide attempt was found for those aged 15 and 19 years, a time when many people have not yet married. Weissman and colleagues reported that there was an increase of twofold to more than sevenfold in the risk for suicide attempts among people who were divorced or separated, compared with those currently married, in 9 countries (28). Therefore, family ties seem to affect the risk for suicide and may be considered a protective factor. Our findings also show that the rate of suicide attempts among single people in Labrador was significantly higher than for the island portion of the province. This might suggest that single people in the Aboriginal population are at greater risk for suicide attempts than are single people in the non-Aboriginal population. Consistent with this, Boothroyd and others, in a study of suicide among Inuit of northern Quebec, reported that 82.8% of Aboriginal people who committed suicide were single (29).

Education

In this study, the rate of suicide attempts was found to be inversely related to the level of education. The rate of suicide attempts among the population with an education level of less than 12 years was higher than among those with 12 or more years of education. This is comparable with findings from other studies (30,31). Our findings also suggest that women with a low level of education are at greater risk for suicide attempts than are men with the same level of education. The greater risk of suicide attempts among those with less education may be related to competitive disadvantage for employment.

Method of Suicide Attempts

In this study, the methods of suicide attempt were primarily "soft" (for example, poisoning), as they are often referred to in the literature (3,24,25,32). Poisoning accounts for only 26% of completed suicides and 83% of attempted suicide in Canada (3). In fact, suicide attempts typically involve less lethal methods than do completed suicides. In this study, male subjects used more lethal methods than did female subjects, which may reflect the seriousness of male subjects' attempts. Lethal methods (for example, hanging and firearms) were used more often in Labrador than in the rest of the province. This may be related to the ease of access to lethal methods (for

example, firearms) in Labrador (a region with a high Aboriginal population), compared with Newfoundland.

Psychiatric Conditions

More than 70% of patients who attempted suicide in this study had associated mental disorders requiring hospitalization. Hospital-based studies have shown that approximately 40% of those who attempt suicide have a history of psychiatric treatment (33). Although a mental disorder diagnosis appears to place a person at increased risk for attempting suicide, this does not imply that mental disorders cause suicide. The diagnosis of a mental disorder is not a sufficient explanation for suicidal behaviour. It is also important to distinguish between the historic experiences and general characteristics of individuals that place them at greater risk for suicide (distal risk factors) and the more immediate risk factors or triggers (proximal risk factors), such as a family breakup or other stressful life events (30). According to our study results, it appears that psychotic conditions (for example, manic depressive psychosis and schizophrenia), personality disorders, and major depressive disorders are the most common distal risk factors for suicide attempts in Newfoundland and Labrador; adjustment reaction and acute reaction to stress are the most frequent proximal risk factors. Adjustment disorder and psychotic conditions were found to be the most common associated mental disorder in female and male subjects, respectively. Dealing with such distal issues as unemployment, poverty, poor education, and lack of opportunity and loss of cultural identity at a societal level may reduce the number of people vulnerable to suicide in the long term (30).

Unfortunately, it is not possible to determine from our findings whether attempting suicide predisposes a person to mental disorders, whether those with mental disorders are especially vulnerable to suicide attempts, or whether there is an underlying predisposition to both mental disorders and attempting suicide. Nonetheless, it is well documented that there is a strong and significant association between attempting suicide and having had a mental disorder prior to, at the same time as, or following an attempt (34).

Our results also show that the rate of associated mental disorders requiring hospitalization in Labrador was 30%, which was significantly lower than the rate for the rest of the province (76%). Malchy and others reported similar findings stating that Aboriginal people who committed suicide were less likely to have received previous psychiatric treatment than their non-Aboriginal counterparts (12). This could be related to the following factors: first, since there are only 2 psychiatrists who travel to Labrador every 2 to 3 months to visit patients, there is limited access to psychiatric services in Labrador, compared with the rest of the province (in case of psychiatric emergencies, patients are transferred to other

health care facilities in the province); second, there is a general tendency for Aboriginal persons, who comprise a large proportion of the Labrador population, not to seek assistance for psychological distress or suicidal ideation (12). The literature suggests alternative explanations, including a lower degree of premeditation among Aboriginal suicide victims and a tendency for Aboriginal people to use such traditional resources as elders or native healers (12). Further investigation into the pattern of help-seeking behaviour among Aboriginal persons prior to suicide is warranted and may provide valuable information on the identification of preventive factors.

Conclusions

The present study identifies a possible increase in the rate of suicide attempts in Newfoundland and Labrador. Of immediate need are tools, methodologies, and training opportunities that will identify vulnerable individuals, the situations or conditions that heighten their vulnerability, and their risk behaviours. Thus health care professionals, as well as friends and family, can be alerted to the imminent danger of suicide. Community members need to be empowered to act with the appropriate resources—within themselves or through access to emergency services—to avert such tragedies.

An increased risk for suicide attempts was associated with single status, female sex, younger age (teen or young adult), and low socioeducational level during the index attempt. The differences between attempted suicide rates in Newfoundland and Labrador and the national level likely reflect social, economic, and cultural factors that can not be fully addressed with hospital data. Further research is needed to explicate these findings and increase our understanding of attempted suicide.

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¹Research Associate, Research and Development Division, NLCHI, St John's, Newfoundland and Labrador.

²Manager, Research and Development Division, NLCHI, St John's, Newfoundland and Labrador.

³Director, Research and Development Division, NLCHI, 1 Crosbie Place, St John's, Newfoundland and Labrador.

Address for correspondence: Dr R Alaghebandan, Research and Development Division, NLCHI, 1 Crosbie Place, St John's, NL A1B 3Y8 e-mail: rezaa@nlchi.nl.ca

Résumé : Tentatives de suicide et facteurs associés à Terre-Neuve et au Labrador, de 1998 à 2000

Objectif : Cette étude examinait l'épidémiologie et les facteurs associés des tentatives de suicide nécessitant une hospitalisation dans la province de Terre-Neuve et du Labrador.

Méthode : Nous avons extrait des données de la base de données provinciale des départs des hôpitaux. Les mesures des résultats comprenaient les taux d'incidence (TI) des tentatives de suicide selon l'âge, le sexe, et la région géographique de résidence. Les données sociodémographiques ont aussi été analysées pour déterminer les facteurs associés.

Résultats : Un total de 978 patients qui ont été hospitalisés en raison de tentatives de suicide ont été identifiés pour les années 1998 à 2000, ce qui a donné un TI global de 68,7 par 100 000 années-personnes (AP). Le taux propre à l'âge pour les personnes de 15 à 19 ans était beaucoup plus élevé, à 143,0 par 100 000 AP. Le ratio global femmes-hommes était de 1,3, le taux de tentatives de suicide étant de 76,1 par 100 000 AP pour les femmes, et de 60,3 par 100 000 AP pour les hommes ($P = 0,001$). Le Labrador (210,2 par 100 000 AP), une région ayant une forte population autochtone, avait un taux plus élevé de tentatives de suicide, comparativement à la portion insulaire de la province (59,0 par 100 000 AP) ($P < 0,001$). Plus de 70 % des hospitalisations étaient associées à un diagnostic psychiatrique. L'empoisonnement était la méthode la plus répandue des tentatives de suicide. Des TI plus élevés de tentatives de suicide ont été constatés chez les personnes qui étaient séparées ou divorcées et chez celles qui étaient moins instruites ($P < 0,001$).

Conclusions : Les tentatives de suicide représentent une préoccupation importante de la santé publique de la province, surtout au Labrador. Un risque accru de tentatives de suicide était associé avec le fait d'être célibataire, femme, adolescent ou jeune adulte, et/ou d'avoir un faible niveau d'instruction, au moment de la tentative répertoriée. Il faut plus de recherche pour expliquer ces résultats et accroître notre compréhension des tentatives de suicide.